

Condition

Shoulder Instability

Definition / Description

clinical condition where the humeral head does not remain centered in the glenoid fossa during shoulder movement. This leads to excessive translation of the humeral head, resulting in pain, partial sliding out, or dislocation.

Populations / Risk Factors

Young male and 15-20

Overhead athletes & Contact Sports

Previous dislocation is the strongest predictor of future instability.

Anatomy: glenoid dysplasia.

Symptoms - (Distribution, description, daily pattern, etc)

Deep shoulder pain of posterior axising, especially in the "apprehension position".

A psychological fear of feeling that the arm will "come out the socket".

Clinical Signs - (Reliable measures)

Releasement test: Relief of apprehension when a posterior force is applied to the humeral head.



Investigations - (Radiology, haematology etc)

x-ray / MRI / CT scan

Atypical Presentations

athlete's "Dead Arm" syndrome: sudden paralyzing pain and weakness in
during a throw, often without a clear dislocation event.

Prognosis

Age factor: Recurrence rates are highly age-dependent.

Surgical: High success rates (80-95%) for traumatic cases with localized labral tears.

Management Options and Levels of Evidence



physiotherapy → strengthening of the rotator cuff

Surgical Repair: Arthroscopic Bankart repair for labral tears or open Latarjet procedure for significant bone loss.

Useful Resources

A/As

↓
orthopaedic surgeons



Condition

Mid portion Achilles Tendinopathy.

Definition / Description

A clinical syndrome characterized by localized tendon pain and swelling. It involves a failed healing response and collagen disorganization, rather than classic inflammation.

Populations / Risk Factors

→ Basketball players / Volleyball p / high jumpers.
→ sudden spike in plyometric volume. limited ankle dorsiflexion.
→ Calf muscle weakness.

Symptoms - (Distribution, description, daily pattern, etc)

→ 2-6cm proximal to the calcaneal insertion.

Pattern: "Morning stiffness" that eases with movement, the "Warm-up effect": pain decreases during exercise but worsens significantly the next morning.

Clinical Signs - (Reliable measures)

Palpation: Localized tenderness and tendon thickening.

~~Single Leg~~

Single-leg Heel Raise: Pain or reduced endurance during repetition.



Investigations (Radiology, haematology etc)

primarily a clinical diagnosis

(US
MRI)

Atypical Presentations

Achilles tendon rupture, Haglund's deformity, Retrocalcaneal bursitis, or systemic inflammatory conditions!

Prognosis

Generally favorable with 12+ weeks of loading
However, jumping athletes have higher risk of recurrence if
return-to-play is rushed before restoring tendon stiffness

Management Options and Levels of Evidence

Useful Resources

RTP: $\left\{ \begin{array}{l} \text{Pain: VAS } < 2/10. \\ \text{Strength: single leg } > 94\% \text{ weight} \\ \text{Plyometrics:} \\ \text{Specialist: No more. (Symptoms)} \end{array} \right.$



Condition	ACL injury.
Definition / Description	A partial or complete tear of the ACL, one of the primary stabilizing ligaments of the knee. Its main function is to prevent anterior translation of the tibia relative to the femur and to provide rotational stability.
Populations / Risk Factors	Demographic : Athletes (soccer, Basketball, Skiing, Football) Mechanisms : Non-contact pivoting, sudden deceleration, or awkward landing. Risk Factors : High "Q-angle", weak core/gluteal.
Symptoms - (Distribution, description, daily pattern, etc)	
Clinical Signs - (Reliable measures)	

Anterior drawer sign
 Lachman's test
 Pivot shift test



look model in → Endonophy

Investigations - (Radiology, haematology etc)

Atypical Presentations

Prognosis

Management Options and Levels of Evidence

Useful Resources

Spanish squat
Nordic hamstring curl.
Single leg landing with to hold.
Figure 8 run.



Condition	<i>Hand injury</i>
Definition / Description	
Populations / Risk Factors	
Symptoms - (Distribution, description, daily pattern, etc)	
Clinical Signs - (Reliable measures)	



